



Amina 18

An 18-year-old female patient, bisexual, of Bosnian origin, from a close-knit to highly symbiotic family system. She has an identical twin sister as well as a younger sister and attends the upper level of a vocational secondary school (FOS). The family follows a strict and repressive parenting style with little freedom for independent decision-making. Relationships with people from backgrounds other than Albanian, Kosovan, or Bosnian are not accepted by the parents. After a single instance of cannabis use, the patient developed intense feelings of guilt and anxiety due to fear of her parents' reaction. The family history reveals a depressive episode in a relative.

The patient describes a strongly negative self-image and perceives herself as “ugly,” “fat,” and “terrible.” Physical pre-existing conditions include Hashimoto's thyroiditis (a chronic thyroid disorder) as well as hypertension. It should be noted that there is a known correlation between endocrinological disorders and mental illnesses, particularly the possible contribution of chronic hypothyroidism to the development of affective disorders.

Comorbid panic attacks (F41.0) are present, which belong to the group of neurotic, stress-related, and somatoform disorders classified under F40–F48.

Diagnosis: F32.1 (G) Moderate depressive episode.

Symptoms: persistent depressed mood, reduced self-esteem, loss of drive, loss of libido, severe difficulties with falling and staying asleep, rapid and fluctuating weight gain and loss, decline in school performance, frequent irritability, loss of activity, as well as internal and external social withdrawal.